

General Practice Accreditation Checklist

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HOW TO USE THIS CHECKLIST

An accreditation survey looks for evidence of everyday practice, not intentions. Work through each criterion, tick only what you can evidence today, and note gaps with an owner and a date. Accreditation surveys currently proceed against the 5th edition of the RACGP Standards; the 6th edition (released 26/08/2026) supersedes it and transition arrangements are yet to be announced, so building evidence against the 6th edition now serves both today's survey and the transition ahead.

Practice name: _____

Suburb: _____

Date completed: _____

NEW new in the 6th edition · **UPD** strengthened or updated · Criteria follow the 6th edition structure (F, CG, PP, CQI).

Foundations of general practice (F1–F11)

Ref	Criterion	What to have ready	Done
F1	Defining and planning for the practice UPD	Current plan covering governance, accountability, strategic and operational planning, and risk management	☐
F2	Response planning	Documented plans for emergencies and business interruptions; staff know their roles	☐
F3	Environmental sustainability and responsibility NEW	Practice approach to sustainability and climate resilience (energy, waste, supply choices)	☐
F4	Induction, training and supporting performance	Induction records for every role, ongoing training matrix, performance reviews on file	☐
F5	Registration and qualifications of practitioners	Current AHPRA registration, qualifications and indemnity insurance for all practitioners	☐
F6	Clinical autonomy of practitioners	Practitioners exercise clinical judgement consistent with professional standards	☐
F7	Practice team culture, safety and involvement	Team can raise ideas and concerns; meetings documented and acted on	☐
F8	Information security UPD	Security policy, access controls, tested backups and an incident response process (test your systems)	☐
F9	Confidentiality and privacy of health and other information	Privacy policy, signed confidentiality agreements, health records held securely	☐
F10	Digital health technologies	Digital tools (My Health Record, electronic prescribing) governed, documented and used safely	☐
F11	Artificial intelligence (AI) NEW	Where AI tools are used: patient transparency, human oversight and risk management documented (applies only where AI is in use)	☐

Clinical governance (CG1–CG13)

Ref	Criterion	What to have ready	Done
CG1	Clinical information systems	Health records complete, legible and audited; clinical software used consistently	☐
CG2	Patient identification	Positive patient identification at registration and before every episode of care	☐
CG3	Facilitating complete patient health records	Records hold history, medicines and allergies; audits run on a schedule	☐
CG4	Provision of clinical and medicines guidelines	Current clinical and therapeutic guidelines accessible to the team and used	☐
CG5	Transitions of care	Referral, handover and discharge processes documented and used	☐
CG6	Follow-up systems	Results, reminders and recalls tracked with a failsafe process	☐
CG7	Managing clinical risks and incidents	Incident register with review, improvement actions and open disclosure	☐
CG8	Immunisations	Schedules, cold chain and registry reporting current	☐
CG9	Infection prevention and control, including reprocessing UPD	IPC policy, risk-based reprocessing of reusable medical devices, staff trained	☐
CG10	Practice environment	Safe, clean premises; hazards, waste and sharps managed	☐
CG11	Practice equipment	Equipment maintained and calibrated to manufacturer instructions; logs current	☐
CG12	Maintaining vaccine potency	Vaccine fridge monitored, twice-daily checks logged, breaches actioned	☐
CG13	Research	Where involved in research, ethics and governance arrangements documented	☐

Patient participation (PP1–PP10)

Ref	Criterion	What to have ready	Done
PP1	Information about the practice	Practice information sheet current (hours, fees, billing, services) in print and online	☐
PP2	Communications	Phone and electronic communication managed safely, including clinical content	☐
PP3	Respectful, culturally appropriate and culturally safe care	Culturally safe care policy; team trained; practice welcoming to Aboriginal and Torres Strait Islander peoples	☐
PP4	Informed consent	Consent processes documented, including procedures, students and observers	☐
PP5	Accessibility of services	Access for people with disability, interpreter use and equitable care for priority groups	☐
PP6	Health promotion and preventive care	Screening and preventive activities delivered and recorded	☐
PP7	Open disclosure and complaints	Complaints register with review, response and an improvement loop	☐

Ref	Criterion	What to have ready	Done
PP8	Engaging consumers UPD	Consumer experience and perspectives feed improvement, beyond collecting feedback	☐
PP9	Responsive system for patient care	Triage, urgent care and continuity-of-care systems work in practice	☐
PP10	Care when the practice is not open	After-hours arrangements safe and matching what patients are told	☐

Continuous quality improvement (CQI1)

Ref	Criterion	What to have ready	Done
CQI1	Continuous quality improvement activities UPD	QI activities driven by practice data, showing how improvement contributes to patient care (now its own standard)	☐

Optional standard: point-of-care testing (PoCT1–PoCT8)

If your practice performs point-of-care testing, the optional PoCT standard also applies: clinical purpose, clinical responsibility, qualifications and training, facilities, performance of tests, data management, quality control and external quality assurance.

Completed by:

Signature:

Date:

This checklist is a general guide and is not exhaustive. Accreditation is assessed by independent surveying agencies against the RACGP Standards in force for your survey. Confirm scope with your accrediting agency. Using this checklist does not guarantee accreditation.